

WEIGHT LOSS

Nutrition, Muscle Preservation & Lifestyle

Protein, resistance training, and the referral boundary of a practice that is not a dietetics clinic

DOCUMENT WL-007	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO All clinical staff	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

Rapid weight loss without adequate protein and resistance training costs lean mass, and lean mass is difficult to regain. This protocol sets the counseling standard, defines what the practice does and does not provide, and identifies when a patient must be referred to a registered dietitian.

KNOW THE BOUNDARY

General nutrition counseling supporting a medical treatment is within reach of most practices. Individualized medical nutrition therapy — particularly for patients with diabetes, renal disease, or a history of disordered eating — is a registered dietitian's scope in many states. Your medical director must define the boundary for your practice and state, and staff must know where it sits.

2 Protein & Intake

Appetite suppression is the mechanism of the drug, which means inadequate protein intake is the default outcome unless it is actively managed. Set a target at initiation rather than responding to lean mass loss later.

Element	Counseling standard
Protein target	An individualized daily target set by the prescriber, expressed in grams and distributed across meals. Your medical director sets the practice framework; body weight and comorbidity determine the individual figure.
Protein first	Eat protein at the start of each meal, before the sense of fullness arrives. This single instruction does more than any other.
Distribution	Spread across the day rather than concentrated in one meal; small, frequent, protein-led eating suits reduced appetite better.
Hydration	A daily fluid target, sipped through the day and away from meals. Dehydration is common and drives much of the fatigue reported.
Fiber	Adequate fiber alongside fluid to manage constipation before it needs medication.
Micronutrients	Assess adequacy where intake is very low. Supplementation decisions sit with the prescriber.
Alcohol	Reduce or avoid. It worsens nausea, dehydration and hypoglycemia risk, and displaces protein calories.

VERY LOW INTAKE IS A CLINICAL FINDING

A patient eating a few hundred calories a day and losing weight rapidly is not a program success. Assess for dehydration, protein inadequacy and lean mass loss, address per this protocol, and consider dose reduction per WL-004. Never congratulate a patient on an intake level that is harming them.

3 Resistance Training

- Resistance training is recommended to every patient at initiation, not introduced after lean mass loss appears.
- Recommend a realistic starting frequency covering major muscle groups, appropriate to the patient's current activity and any physical limitation.
- Progressive overload matters more than modality — body weight, bands, machines or free weights are all acceptable starting points.
- Combine with general activity guidance; steps and daily movement support adherence but do not substitute for resistance work.
- Refer to a qualified exercise professional where the patient has cardiac, orthopedic or mobility considerations.
- Record the recommendation given and the patient's response at each review.

FRAMING MATTERS

Patients hear resistance training as an aesthetic suggestion. Frame it as part of the treatment: the medication reduces intake, and training is what determines whether the weight lost comes from fat or from muscle. Patients act on that framing far more often.

4 Tracking

Measure	Interval	Why
Weight	Every visit	Trajectory against the agreed goal.
Body composition where available	At baseline and at set intervals	Distinguishes fat loss from lean mass loss. Scale weight alone cannot.
Circumference measurements	At baseline and at set intervals	Simple, reproducible, and useful when composition analysis is unavailable.
Protein intake	Every visit	The variable most likely to be inadequate.
Resistance training frequency	Every visit	Adherence to the recommendation given.
Functional markers	Periodically	Strength, stair climbing, grip. Falling function during weight loss is a warning sign.

SCALE WEIGHT IS A POOR SOLE ENDPOINT

A program that reports only scale weight cannot distinguish an excellent outcome from a harmful one. Where body composition analysis is unavailable, use circumference and functional markers alongside weight rather than weight alone.

5 Referral & Scope Boundaries

Refer to a registered dietitian

- Any positive or equivocal disordered eating screen — before continuing therapy, per WL-003.
 - Persistent inability to meet protein targets despite counseling.
 - Diabetes requiring coordinated medical nutrition therapy.
 - Renal impairment, hepatic disease, or another condition with specific dietary requirements.
 - Prior bariatric surgery.
 - Pregnancy occurring during the program — therapy is stopped and the patient referred.
 - Evidence of lean mass loss or falling functional capacity.
 - Any patient requesting a detailed individualized meal plan, where that exceeds your practice's defined scope.
- ☐ A named registered dietitian or referral pathway identified and current.
 - ☐ Referral criteria known to all clinical staff.
 - ☐ Referrals made in writing and recorded, with outcomes followed up.
 - ☐ Staff clear on what nutrition guidance they may and may not provide.
 - ☐ Written patient education materials reviewed and approved by the medical director.

6 Quick Reference

QUICK REFERENCE CARD

Every weight-loss visit

- Ask the protein number.** Not 'are you eating well?'
- Protein first at every meal** — the highest-yield instruction you can give.
- Check hydration and fiber.** Prevents most constipation.
- Resistance training** — frame it as part of the treatment, not a suggestion.
- Track more than the scale.** Composition, circumference, function.
- Very low intake is a finding**, not a success. Act on it.
- Refer** for eating disorder signals, diabetes, renal disease, or lean mass loss.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

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